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Name: رشا سيد هليل حسانين National ID: 28309102103889	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



رشا
ميد هليل حسنين
قرية منيل السلطان
مركز اطفح - الجيزة



٢٨٣٠٩١٠٢١٠٣٨٨٩
٢٨٣٠٩١٠٢١٠٣٨٨٩
٢٨٣٠٩١٠٢١٠٣٨٨٩



٢٠٢٥/١١
٢٨٣٠٩١٠٢١٠٣٨٨٩
حاصله على دبلوم صناعي

متزوجة
أنثى
مسلمة
محمود احمد احمد على عياد
البطاقة سارية حتى ٢٠٣٢/١١/٠٥





Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



A-Patient ID:

1- Name: رشابيد خليل حستين

Sex:

MDT form

3- Medical numbers

Female

83218382

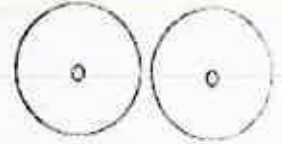
Old Med:

Old Med:

2192-2024

2- ID numbers:

Patient Age: 41 Y



Family history

Family history
+ve

Phone

Phone
01282133656

BMI

Wt

REASON OF MDT

STANDARD

Marital status

Marital status
+ve

NRS Score

NRS Score

GROUP

GROUP

Date of MDT

03/10/2024

Comorbidity

Comorbidity
+ve

Menopausal status

Menopausal status
+ve

Initial STAGE

Initial STAGE

03/10/2024

Breast Imaging:

rt enhanced mass, lt axillary LN

Clinical data

Mets Workup:

pet ct: FREE

BX
IDC

ER

PR

Her 2 Neu

SISH

Ki67
30

ER comment

ER comment
+ve

PR comment
+ve

Her 2 Neu comment
+ve

SISH comment

T

2

N

0

M

0

Sonomammography

PATIENT HISTORY

rt skin sparing mastectomy+ALND+LD FLAPS:IDC GII with focal DCIS PT2(3CM), free deep margins, nipple, areola, LN 0/18

MDT OUTCOME & PLAN

- 1- ADJUVANT TAXOTERE ENDOXAN X4, ZOLADEX
- 2- ADJUVANT AI ZOLADEX

SIGNATURE

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

Signature

RADIOLOGY

RADIOLOGY

SURGERY

SURGERY

PATHOLOGY

PATHOLOGY

Psychiatric

Psychiatric

Nutrition

Nutrition

Name: رشا سيد هليل حسنين		
MR. : 5667		
Detailed Medical Report		
		السيد
تحيه طيبة وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه The patient was prescribed ARAMIDEX CALCIUM with a dosing schedule of 1 tablet daily (1*1*90) and received a single dose of ALPHA ZOLADEX 10.8 mg subcutaneously. Subsequently, the patient was noted to be on an aromatase inhibitor (AI) regimen with ZOLADEX, and the plan was to continue both ARAMIDEX and ZOLADEX for a duration of three months. The current plan is to maintain this therapy as outlined.		
تحريرا في ,		
	التاريخ :	18-09-2026
	الوقت :	ايه مدحت بدوي محمد
الطبيب المعالج :		
يعتمد ,		



Patient Name : الأستاذة / رشا سيد هاني حسنين
Patient No. : 28309102103889
Path No. : SR-953/24
Sex : Female
Age : 40 Y
Referring Doc. :

Request No. : 13003208
Request Date : 15/07/2024
Verification Date : 03/08/2024
Financial Category : اورام دار السلام - نفقة الدولة
Department : Out Patient
IHC No. :
Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast mass

Nature of Specimen: Block revision

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

- * IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 95% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 95% of tumor cells (score 8/8)..
- Her2: Faint incomplete membrane reaction in >10% of tumor cells (score 1+).
- Ki67: 30% nuclear reaction of tumor cells.

Conclusion:

Right breast mass, block revision:

Invasive duct carcinoma, Grade II.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Negative (score 1+).
- Ki67: Positive in 30% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between

Verified by :

DR.Israa Elgendy

Printing Date : 04/08/2024 08:47:48

Page 1 of 2

Printed By : marwa mohamed yousse



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Room No. (4) - First Floor-Building No, 987- Nile Corniche - Mixr El Qadima - Cairo, Egypt



Patient Name الأمتلاء / رشاميد هليل حسنين
Patient No. 28309102103889
Path No. SR-953/24
Sex Female
Age 40 Y
Referring Doc.

Request No. 13003208
Request Date 15/07/2024
Verification Date 03/08/2024
Financial Category أورام ناز السلام - نفقة الدولة
Department Out Patient

Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, Intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"

Reference; ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Walaa Ghoneim
Dr. Nermin Mohamed

Verified by :
DR. Israa Elgendy

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mizr El Qadema - Cairo, Egypt

Patient Name : الأستاذة / رشا عبد الحليم جبريل
Patient No. : 28309102103889
Path No. : 3098/24
Sex : Female
Age : 40 Y
Referring Doc :

Request No. : 13005148
Request Date : 27/08/2024
Verification Date : 17/09/2024
Financial Category : نفقة الدولة
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Right breast cancer, Up-Front

Nature Specimen: Right skin sparing mastectomy and Axillary lymph node dissection.

Gross Examination:

- Procedure: Right skin sparing measured 16x15x5 cm with partial skin cover measured 7x3 cm.
- Tumor extent: A grayish white firm mass.
- Size: 3 x 2.5 x 2 cm. *مكتبة*
- Skin involvement: Nipple & areola were unremarkable grossly.
- Distance from margins: 3.5 cm away from deep margin.
- Lymph nodes number and size: 18 L.Ns up to 2.5x1 cm.

Microscopic Examination:

- Histological type: Invasive duct carcinoma.
- Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- Ductal carcinoma in situ: Focal.
- Skin involvement: Free Nipple and areola.
- Lymph node involvement: Free (0/18).
- Resection Margins: Free deep margin.

Diagnosis:

Right breast cancer Up-Front, Skin sparing mastectomy and axillary dissection specimen:
Invasive duct carcinoma, Grade II with focal DCIS.

Free deep margin.

Free nipple and areola.

Free lymph nodes (0/18).

Stage: pT2, pN0.

Verified by :

Dr. Sagda El-Kady

Printing Date : 17/09/2024 11:31:38

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Printing By : manwa mohamed youssef

CAP
ACCREDITED

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadima - Cairo, Egypt

Dr. Mohamed El-Sayed El-Mahallawi
Date: 12/07/2024

¹⁸F- FDG WHOLE BODY PET/CT EXAMINATION

History:

55-year-old female patient, presented with recently diagnosed breast cancer, referred for further PET/CT assessment.

Examination:

62.05 MBq ¹⁸F-FDG was administered to the patient and images after 60 minutes.
Glucose level was 87 mg/dl.
A contrast examination was taken after IV non-ionic contrast administration of the same regions. Images of CT and corresponding functional PET images were taken in axial, coronal and sagittal planes.

PET/CT FINDINGS:

Head and Neck:

No evidence of any abnormally hypermetabolic lymph nodes.
The visualized portions of the brain exhibited normal FDG biodistribution.
No abnormal FDG uptake is seen in the oropharynx, salivary glands and larynx.
No parathyroid/thyroid focal lesions could be elicited.

Chest:

Hypermetabolic irregular speculated right breast soft tissue mass lesion is seen by the PET with non FDG avid areolar skin thickening, measuring 47- 26.5x16x21 mm and achieving up to 6.45 SUVmax.

No other non FDG soft tissue lesions detected.
No FDG avid hilar lymphadenopathy, the distal or hilar lymphadenopathy.

Multiple bilateral non FDG avid small lymph nodes, likely



- reactive.
- No metabolically active pulmonary nodules detected.
 - Non-FDG avid pulmonary nodule is seen at left lung lower lobe, based on oblique fissure, measuring about, about 9.1x9.7 mm.
 - Other tiny calcified nodule is seen at left lung upper lobe, mostly of old granulomatous nature.
 - No pleural or pericardial effusion seen.
 - Normal tracer biodistribution uptake is noted in the myocardium.



Abdomen & Pelvis:-

- There is normal distribution of the radiotracer within the gastrointestinal and genitourinary systems without focal areas of abnormal metabolism.
- Physiological uptake in liver, adrenal glands, spleen and pancreas.
- Physiological excretion of FDG in the kidneys.
- No ascites or hypermetabolic lymphadenopathy noted.

Musculoskeletal:-

- No metabolically active osseous lesions could be detected.

CONCLUSION:-

- Hypermetabolic right breast soft tissue lesion, as described, mostly of malignant neoplastic nature, for histopathological assessment.
- Small inactive left lung lower lobar nodule, could be metastatic, for follow up.

Much Obligated,

Head Of Radiology Department
Dr. Ehab Harby

Head of PET/CT unit
Dr. Ahmed A. Elazeem

General supervisor of radiology departments
Dr. Haltham A. Elghafar, MD

Consultants
DR. YASMINE ATTEYA, MD
PROF. DR. SUSAN ADIL